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Nursing quick reference

A clearer summary page to sit alongside the original booklet. This keeps the same content focus, but strips it back to the things student nurses most need to recognise, say, and do.

ELLIE AT A GLANCE

10-month-old with bronchiolitis, **RR 62/min**, **SpO₂ 90% on air**, recession, grunting, poor feeding, and weight loss. For a student nurse, that combination points to **increased work of breathing + hypoxaemia + feeding risk**, so this child needs escalation, supportive care, and close reassessment.

RR 62/min

SpO₂ 90% on air

Recession

Grunting

Poor feeding

Weight loss

RECOGNISE

Look beyond “wheeze.” In bronchiolitis, the most important clues are **work of breathing**, **oxygenation**, **fatigue**, and **feeding**.

EXPLAIN

Use mechanism language in answers: inflammation and mucus narrow small airways, air movement worsens, ventilation drops, and the infant works harder to maintain oxygenation.

ESCALATE

Grunting, significant recession, worsening fatigue, apnoea, or persistent low saturations are not “watch and wait” features. Escalate early.

WHAT TO SAY IN AN EXAM

WHY IT MATTERS

NURSING ACTION

“This is mainly a supportive-care illness.”

Shows you know bronchiolitis is usually viral, so management is not routine antibiotics.

Support oxygenation, hydration, feeding, and observations.

“Poor feeding is clinically important.”

Infants need coordinated suck-swallow-breathe, which breaks down in distress.

Track intake, wet nappies, weight, and discuss NG/fluids if needed.

“Grunting is a red flag.”

It suggests the child is using intrinsic PEEP to keep alveoli open.

Escalate, reassess A–E, and do not delay senior review.

“Parent concern matters.”

Family distress can be an early warning that the child is worsening.

Listen, explain clearly, and act on deterioration concerns early.

COMMON STUDENT MISTAKE

Describing signs without saying what they mean. A stronger answer links the sign to physiology and then to what you would do next.

USEFUL SENTENCE STEM

“I’m concerned because this infant has signs of worsening respiratory distress, poor feeding, and hypoxaemia, so my priorities are escalation, supportive care, and ongoing reassessment.”

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Other common respiratory conditions

A breathless child is not automatically “just bronchiolitis”. This page gives nursing students a quick way to separate the common paediatric patterns they are likely to see on placement or in an exam.

CONDITION	PATTERN CLUES	NOISE / AIRWAY LEVEL	WHAT NURSES SHOULD THINK ABOUT
Bronchiolitis	Usually infant, coryza, cough, poor feeding, recession, wheeze or crackles	Mostly lower small-airway problem	Supportive care, oxygenation, hydration, feeding, escalation if tiring
Croup	Barking cough, hoarse voice, worse at night	Stridor , so think upper airway narrowing	Keep child calm, avoid upsetting them, watch for stridor at rest or severe distress
Viral wheeze / asthma	Expiratory wheeze, prolonged expiration, breathlessness, often previous similar episodes	Lower airway bronchospasm / wheeze pattern	Check inhaler/spacer response, action plan, and local acute wheeze pathway
Pneumonia	Fever, cough, increased WOB, focal crackles, sometimes focal chest findings	Infective lung/parenchymal problem rather than classic bronchiolitis pattern	Think infection, hydration, sepsis awareness, antibiotics when clinically indicated

PATTERN RECOGNITION

- **Bark + stridor** pushes you towards croup.
- **Expiratory wheeze + prolonged expiration** fits wheeze or asthma more than bronchiolitis.
- **Infant + coryza + poor feeding + recession** keeps bronchiolitis high on the list.
- **High fever + focal crackles** makes pneumonia more likely.

ESCALATE BEFORE YOU OVER-LABEL

If the child is blue, apnoeic, drooling, increasingly drowsy, or failing to maintain oxygenation, the immediate priority is airway and breathing support, not trying to win the diagnosis in one sentence.

Explain simply

“We’re watching how hard the breathing looks, not just whether there is a cough or wheeze.”

Name the worry

“The concern is the work of breathing, oxygen level, and whether your child can keep feeding.”

Say what you are doing

“We’re repeating observations, supporting breathing, and escalating early if things worsen.”

Safety-net clearly

“If breathing gets noisier, feeding drops, or your child becomes more sleepy, tell us straight away.”